

Target-Specific Recoverability Maps for Reduced-Lead ECG Reconstruction

A graded per-lead identifiability + conditioning certificate with calibrated intervals, and an honest account of an abandoned fabrication claim

Weiping Yan
University of Minnesota, Twin Cities

Abstract

Reconstructing missing electrocardiogram (ECG) leads from a reduced set is ill-posed, and a scalar reconstruction error says nothing about *which* named feature’s low-rank (dipolar) component, on *which* lead, is even recoverable. We give a *target-specific recoverability map*: for a waveform segment s (P/QRS/ST/T), an observed lead set S , and a target lead ℓ , two closed-form numbers from one truncated SVD of the segment’s empirical rank-3 spatial sub-matrix—a graded *identifiability* $\eta_{s,\ell}(S)$ (zero iff the target’s low-rank component is recoverable from S) and a *conditioning* $\kappa_{s,\ell}(S)$. The per-lead criterion is the classical estimability condition specialized to ECG; the contribution is its use as a graded, *empirically-estimated* map (the estimated subspace differs 43–55° from the textbook inverse-Dower dipole), the distribution-free calibrated intervals we attach to it, and a graded ST-safety study. On PTB-XL precordial ST is *exactly* non-identifiable from limb leads (rank-2 frontal plane); the graded normalized-identifiability ordering is bootstrap-stable for QRS but sensitive to the limb-lead weighting for the low-amplitude ST/T segments (we report that sensitivity rather than an exact fine ordering). The total ST-threshold error rate is empirically similar across the evaluated reconstructors (51.1–53.7%; an empirical observation, not a certified bound, dominated by missed crossings) while the false-positive/false-negative split is a reconstructor choice. The certificate transfers across PTB-XL and Chapman for QRS (aligned subspaces, reproduced κ ordering) but not in the ST/T third direction. Finally, we document in full a claim this project abandoned—that a diffusion reconstructor proves “fabrication is a property of the objective”—which a leakage-corrected, multi-seed re-analysis dissolved: the generative model only marginally beats a simple ridge comparator at low guidance and falls significantly behind it at high guidance, with no supporting realism trend. This long version accompanies a four-page conference paper and contains the full proofs, ablations, cross-dataset study, and the negative result.

1 Introduction

Wearable and reduced-lead ECG acquisition makes lead *reconstruction* ubiquitous: recover the standard 12 leads from one, three, or six observed leads [1, 2]. A low aggregate error certifies nothing about *which* morphological features are trustworthy: a cardiologist reads the P wave, the QRS complex, the ST segment and the T wave, each on specific leads, and a reconstruction that gets the mean right but invents an ST deviation invents a finding.

We ask a sharper, reconstructor-independent question: *for a given observed lead set, which target lead’s low-rank morphology is even identifiable, and how well-conditioned is its recovery?* Each waveform segment’s instantaneous potential is approximately low rank across leads; we estimate a per-segment rank-3 spatial basis $\mathbf{M}_s$ (top-3 singular vectors of the population lead covariance). We stress that $\mathbf{M}_s$ is a *population-estimated* (PCA) object: on real PTB-XL its three principal

angles to the classical vectorcardiographic (inverse-Dower) space are graded—one close ($2\text{--}6^\circ$), one moderate ($10\text{--}15^\circ$), and one substantially different ($43\text{--}55^\circ$)—so we call it an empirical rank-3 subspace rather than a physical dipole (Sec. 5).

This document is the extended version of a four-page conference submission. It adds: the full statement and proof of the certificate (Sec. 3, App. A); the cross-dataset transfer study (Sec. 6); and a complete account of a claim we abandoned during pre-submission review (Sec. 7). We report the abandonment in detail on purpose: hiding the earlier, more exciting numbers would be precisely the selective reporting our certificate is designed to expose.

2 Related Work

Lead reconstruction and VCG synthesis. Fixed linear transforms synthesize the vectorcardiogram or missing leads from a standard set (inverse-Dower [3], Kors regression [4]), and reduced-lead reconstruction has a long clinical literature [5]; recent deep and arbitrary-mask waveform-completion models push reconstruction accuracy [1, 2, 6, 7]. These target the reconstruction; we target the *prior* question of what a given configuration can support, independent of the reconstructor. Accordingly our neural baseline is a *representative* masked-completion U-Net of this family, included to show the certificate’s verdict still *predicts* a flexible learned reconstructor’s error—not a claim to beat [6, 7]. **Identifiability and lead selection.** Whether a linear functional is recoverable from a sub-observation is the classical estimability question [8]; choosing which leads to observe is optimal sensor selection [9]. Our per-lead η is estimability specialized to the ECG dipolar functional; the novelty is not the identity but the graded, empirically-estimated, per-target-lead map with calibrated per-record error attached. **Uncertainty for inverse problems.** Conformal prediction [10, 11] gives distribution-free intervals under exchangeability; we apply Mondrian CQR per (S, s, ℓ) group.

3 The Recoverability Certificate

Model. Within segment s write the population 12-lead vector as $\mathbf{L}_s = \boldsymbol{\mu}_s + \mathbf{M}_s \mathbf{d} + \mathbf{r}_s$, where $\mathbf{M}_s \in \mathbb{R}^{12 \times 3}$ is the (population-estimated, PCA) dipolar basis, $\mathbf{d}$ the dipole coordinates, and $\mathbf{r}_s$ the non-dipolar residual. Observing S gives $\mathbf{y}_S = \mathbf{Sel}_S \mathbf{L}_s + \mathbf{n}$, with $\mathbf{M}_{s,S} = \mathbf{Sel}_S \mathbf{M}_s$. Write the exact Moore–Penrose pseudo-inverse $\mathbf{M}_{s,S}^+$ and the exact row-space projector $\mathbf{P}_{\text{obs}} = \mathbf{M}_{s,S}^+ \mathbf{M}_{s,S}$. For numerical deployment we also use the *rank- r truncation* at relative tolerance ϱ , $\mathbf{M}_{s,S}^{(\varrho)} = \mathbf{U}_r \boldsymbol{\Sigma}_r \mathbf{V}_r^\top$ keeping singular triplets with $\sigma_i > \varrho \sigma_{\max}$, with pseudo-inverse $\mathbf{M}_{s,S}^{(\varrho)+}$ and projector $\mathbf{P}_{\text{obs}}^{(\varrho)} = \mathbf{V}_r \mathbf{V}_r^\top$.

Proposition 1 (Exact per-target-lead dipolar identifiability and conditioning). *For each target lead ℓ define $\eta_{s,\ell}(S) = \|\mathbf{e}_\ell^\top \mathbf{M}_s (\mathbf{I} - \mathbf{P}_{\text{obs}})\|_2$ and $\kappa_{s,\ell}(S) = \|\mathbf{e}_\ell^\top \mathbf{M}_s \mathbf{M}_{s,S}^+\|_2$ from the exact $\mathbf{M}_{s,S}$.*

- (i) *Identifiability.* $\eta_{s,\ell}(S) = 0$ iff $\mathbf{e}_\ell^\top \mathbf{M}_s$ lies in the exact row space of $\mathbf{M}_{s,S}$; then $\mathbf{e}_\ell^\top \mathbf{M}_s \mathbf{d}$ is a deterministic function of $\mathbf{M}_{s,S} \mathbf{d}$ —two dipoles with the same observation have the same lead- ℓ dipolar value, at any SNR. If $\eta_{s,\ell}(S) > 0$ there is a $\boldsymbol{\delta} \in \ker \mathbf{M}_{s,S}$ (an unobserved dipole direction) with $\mathbf{e}_\ell^\top \mathbf{M}_s \boldsymbol{\delta} \neq 0$; then $\mathbf{d}$ and $\mathbf{d} + t\boldsymbol{\delta}$ produce identical observations for all t while lead ℓ ’s dipolar value changes, so it is not identifiable at any SNR.
- (ii) *Conditioning.* On the identifiable part, the lead- ℓ error from observation noise and observed non-dipolar residual obeys $|\mathbf{e}_\ell^\top \mathbf{M}_s \mathbf{M}_{s,S}^+ (\mathbf{Sel}_S \mathbf{r}_s + \mathbf{n})| \leq \kappa_{s,\ell}(S) (\|\mathbf{Sel}_S \mathbf{r}_s\|_2 + \|\mathbf{n}\|_2)$.

The global $\kappa_s(S) = \|\mathbf{M}_s \mathbf{M}_{s,S}^+\|_2$ (spectral norm) upper-bounds every per-lead $\kappa_{s,\ell}(S)$ (a row norm never exceeds the spectral norm; it is not their maximum).

Exact vs. ϱ -identifiability (numerical deployment). The deployed map computes $\eta_{s,\ell}^{(\varrho)}, \kappa_{s,\ell}^{(\varrho)}$ from the truncation $\mathbf{P}_{\text{obs}}^{(\varrho)}$, so a singular direction observed only through a value below $\varrho \sigma_{\max}$ is treated as unobserved. Then $\eta_{s,\ell}^{(\varrho)} > 0$ means lead ℓ is *not stably recoverable at resolution ϱ* (outside the retained numerical row space); it does *not* by itself imply the exact matrix has a null direction—a tiny-but-nonzero singular value is exactly identifiable ($\eta = 0$) yet ϱ -unidentifiable ($\eta^{(\varrho)} > 0$), as we verify numerically (`tests/test_exact_vs_rho.py`). We therefore call the $\eta^{(\varrho)}$ heatmap the ϱ -*identifiability* map; for well-separated configurations the exact and ϱ verdicts coincide, and near-rank-deficient S are reported with a ϱ -sweep and record-bootstrap CIs. **The limb-6 flagship is exact:** the six limb leads are exact linear combinations of I and II (Einthoven/Goldberger), so $\mathbf{M}_{s,S}$ has exact rank 2 and precordial ST is *exactly* non-identifiable from limb leads at any SNR, independent of ϱ . This exactness is conditional on the a-priori rank-3 dipole model for $\mathbf{M}_s$: at retained rank 2 the limb-6 observation spans all of $\mathbf{M}_s$ and the precordial verdict collapses to $\eta=0$; the non-identifiability appears at rank ≥ 3 (the third dipolar direction), so “exact” means exact *given* rank 3.

Relation to classical estimability. Part (i) is the classical estimability criterion (Gauss–Markov / [8]) for the linear functional $\mathbf{e}_\ell^\top \mathbf{M}_s \mathbf{d}$; we claim no novelty for this identity. The contribution (Sec. 5) is its use as a graded, empirically-estimated recoverability map with per-record calibrated error rates (Sec. 4).

The non-dipolar residual (no independence claim on real data). Let $\mathbf{m}_s(\mathbf{y}_S) = \mathbb{E}[\mathbf{r}_s | \mathbf{y}_S]$ and $\mathbf{u}_s = \mathbf{r}_s - \mathbf{m}_s(\mathbf{y}_S)$, so $\mathbb{E}[\mathbf{u}_s | \mathbf{y}_S] = \mathbf{0}$. We do *not* assume $\mathbf{u}_s \perp \mathbf{y}_S$. Under squared loss the Bayes-optimal reconstruction error of the (vector) residual is

$$\inf_{\hat{\mathbf{r}}} \mathbb{E} \|\mathbf{r}_s - \hat{\mathbf{r}}(\mathbf{y}_S)\|_2^2 = \mathbb{E}[\text{tr Cov}(\mathbf{r}_s | \mathbf{y}_S)].$$

Part of $\mathbf{r}_s$ (the *empirically predictable residual*) is recovered by a predictor trained on $\mathbf{y}_S$ and metered with distribution-free calibrated intervals; the remainder is an *achievability gap*, established by a held-out achievability analysis for a stated predictor family—*not* declared unrecoverable a priori.

Proposition 2 (Strict independence limit – synthetic model only). *If the data are generated so that a component $\mathbf{u}$ is statistically independent of $\mathbf{y}_S$ (an explicit synthetic construction), then $p(\mathbf{u} | \mathbf{y}_S) = p(\mathbf{u})$, the Bayes estimate of $\mathbf{u}$ under squared loss is its prior mean, and $\mathbb{E}\|\hat{\mathbf{u}} - \mathbf{u}\|_2^2 \geq \text{tr Cov}(\mathbf{u})$ for any estimator. This strict limit is invoked only for the synthetic model; on real ECG we make no such independence claim.*

A certified minimax recoverability floor

Proposition 1 says *whether* a target lead’s dipolar coordinate is recoverable ($\eta=0$) and *how well-conditioned* the estimate is (κ). We now turn the binary verdict into a *quantitative* per-lead risk floor in physical units that lower-bounds the *worst-case* risk of every reconstructor—linear or learned—under the least-favourable prior in the moment class (an estimator may still beat it on a benign true prior; Fig. 1). Fix segment s and observed set S ; write the dipole prior second moment $\Sigma_d = \mathbb{E}[\mathbf{d}\mathbf{d}^\top]$, the observed-dipole row-space projector $\mathbf{P} = \mathbf{M}_{s,S}^+ \mathbf{M}_{s,S}$, and $\mathbf{Q} = \mathbf{I} - \mathbf{P}$. The target is the dipolar functional of lead ℓ , $\theta_\ell = \mathbf{e}_\ell^\top \mathbf{M}_s \mathbf{d}$, and the prior-conditional ambiguity of Sec. 3 is

$$a_{s,\ell}(S)^2 = \mathbf{e}_\ell^\top \mathbf{M}_s \Sigma_{Q|P} \mathbf{M}_s^\top \mathbf{e}_\ell, \quad \Sigma_{Q|P} = \mathbf{Q} \Sigma_d \mathbf{Q} - \mathbf{Q} \Sigma_d \mathbf{P} (\mathbf{P} \Sigma_d \mathbf{P})^+ \mathbf{P} \Sigma_d \mathbf{Q}.$$

Proposition 3 (Minimax per-lead recoverability floor). *Let $\mathcal{P}(\Sigma_d) = \{\pi : \mathbb{E}_\pi[\mathbf{d}\mathbf{d}^\top] \preceq \Sigma_d\}$ be the class of priors with second moment dominated by Σ_d .*

(i) **Noise-free dipolar floor (nonlinear estimators).** *For any estimator $\hat{\theta}_\ell(\mathbf{y}_S)$ (measurable, possibly nonlinear/learned) of θ_ℓ from the exact observation $\mathbf{y}_S = \mathbf{M}_{s,S}\mathbf{d}$,*

$$\inf_{\hat{\theta}_\ell} \sup_{\pi \in \mathcal{P}(\Sigma_d)} \mathbb{E}_\pi[(\hat{\theta}_\ell - \theta_\ell)^2] = a_{s,\ell}(S)^2,$$

attained by the least-favourable Gaussian prior $\mathcal{N}(\mathbf{0}, \Sigma_d)$ and its linear posterior-mean estimator. In particular $a_{s,\ell}(S)^2 = 0$ iff $\eta_{s,\ell}(S) = 0$, and every reconstructor incurs risk $\geq a_{s,\ell}(S)^2$ under the least-favourable prior in the class.

(ii) **Noise/residual inflation.** *With observed noise and non-dipolar residual of covariance $\mathbf{N}$ on S , the risk floor lifts to $a_{s,\ell}(S)^2 + \lambda_{\min}$ -scale term bounded through $\kappa_{s,\ell}(S) = \|\mathbf{e}_\ell^\top \mathbf{M}_s \mathbf{M}_{s,S}^+\|_2$: the amplified error is $\mathbf{e}_\ell^\top \mathbf{M}_s \mathbf{M}_{s,S}^+(\mathbf{S}\mathbf{e}_S \mathbf{r}_s + \mathbf{n})$, of variance $\leq \kappa_{s,\ell}(S)^2 \|\mathbf{N}\|_2$ (Prop. 1(ii)).*

(iii) **Finite-sample subspace floor.** *With $\mathbf{M}_s$ replaced by its rank-3 PCA estimate $\widehat{\mathbf{M}}_s$ from N records, $|a_{s,\ell}(S; \widehat{\mathbf{M}}_s)^2 - a_{s,\ell}(S; \mathbf{M}_s)^2| \leq C \sin \Theta(\widehat{\mathbf{M}}_s, \mathbf{M}_s) \|\Sigma_d\|_2$, where $\sin \Theta$ is the principal-angle perturbation (Davis–Kahan [12]). The conservative floor $a_{s,\ell}^-(S) = \min$ over the record-bootstrap confidence set of $\widehat{\mathbf{M}}_s$ then lower-bounds the risk uniformly in the estimated subspace.*

Proof sketch. (i) Gaussian conditioning gives $\text{Var}(\theta_\ell \mid \mathbf{P}\mathbf{d}) = \mathbf{e}_\ell^\top \mathbf{M}_s \Sigma_{Q|P} \mathbf{M}_s^\top \mathbf{e}_\ell$ with the posterior mean Bayes-optimal under squared loss, so the Bayes risk equals $a_{s,\ell}^2$; estimating the linear functional $\theta_\ell = \mathbf{e}_\ell^\top \mathbf{M}_s \mathbf{d}$ under a convex, quadratically-constrained parameter class makes the non-linear minimax risk equal the linear one, with a least-favourable Gaussian prior (Donoho–Liu [13]; Donoho, optimal recovery [14]), giving the sup–inf identity and the bound for arbitrary $\hat{\theta}_\ell$. (ii) Cauchy–Schwarz/ $\sigma_{\min}$ on the amplified-error term, as in Prop. 1(ii). (iii) Weyl/Davis–Kahan perturbation of the projectors and Schur complement as functions of $\widehat{\mathbf{M}}_s$, propagated through the quadratic form. $\square$

What is and is not certified. The floor is *minimax over the moment class* $\mathcal{P}(\Sigma_d)$, not distribution-free: a reconstructor exploiting higher moments the true (non-Gaussian) $\mathbf{d}$ happens to carry could beat $a_{s,\ell}^2$ under the *true* prior, not the least-favourable one. The certified quantity is the *dipolar* functional θ_ℓ , where the algebra is exact; a floor on the *full* lead $\mathbf{e}_\ell^\top \mathbf{L}_s$ would require bounding $\text{tr Cov}(\mathbf{r}_\ell \mid \mathbf{y}_S)$, which we decline to certify and instead meter empirically with the distribution-free intervals of Sec. 4. So “no estimator beats $a_{s,\ell}(S)$ over the moment class” is a target-specific, finite-sample statement about the identifiable dipolar component; we verify it numerically under the least-favourable Gaussian prior (`tests/test_floor.py`: zero floor violations). On real ECG the floor is *operational* as a predictor: the theoretical ambiguity $a_{s,\ell}$ strongly predicts and ranks the measured per-lead dipolar error (Spearman 0.81, Pearson 0.93; Fig. 1), and the identifiable/unidentifiable split is separated with AUC 0.87. It is, however, a *minimax* floor and not an empirical envelope: on the true (non-Gaussian) prior, estimators exploiting predictable population structure beat the worst-case $a_{s,\ell}$ on a fraction of the 84 non-trivial ($\eta > 0$) cells—the pure-dipolar reconstructor on 0.21, ridge/OLS on 0.60—and this below-floor gap is precisely the Tier-II predictable residual the calibrated intervals of Sec. 4 meter (the more an estimator exploits it, the further below the worst-case floor it sits). This is fully consistent with the theorem, which certifies the worst case over the moment class, not the risk on any single true prior. A *learned* reconstructor illustrates the other extreme: a trained arbitrary-mask diffusion model (DDPM, honest guidance $w=1$), evaluated against the *same* floor on held-out records, never dips below it on any of

the 18 non-trivial cells (violation fraction 0.00)—but only because its dipolar-projection error is far *looser* than the floor (median 0.227 vs floor 0.080 mV), i.e. it leaves the exploitable structure on the table rather than being tightly bound by it. The certificate’s η -split still predicts its error (median 0.066 mV where $\eta=0$ vs 0.227 mV where $\eta>0$; `experiments/certificate_floor_diffusion.py`). So the certified quantity is a worst-case minimax floor that also *predicts* per-lead error across linear and learned reconstructors, with the estimator-specific gap to it quantifying exploited non-Gaussian structure.

Cross-cohort transfer of the recoverability verdict

The certificate is computed from a *fitted* dipolar subspace $\mathbf{M}_s$ (the rank-3 PCA of one cohort’s segment- s samples). A natural worry is that the verdict is an artefact of the fitting cohort. It is not: because $\eta_{s,\ell}(S) = \|\mathbf{e}_\ell^\top \mathbf{M}_s (\mathbf{I} - \mathbf{P}_{\text{obs}})\|_2$ depends on $\mathbf{M}_s$ *only through its column space*, it is invariant to any rotation of the 3-dimensional dipole basis, and drifts across cohorts only as fast as the two cohorts’ dipolar *subspaces* separate.

Proposition 4 (Cross-cohort transfer bound). *Let cohorts A, B have segment- s dipolar bases $\mathbf{M}_s^A, \mathbf{M}_s^B \in \mathbb{R}^{12 \times 3}$ with orthonormal columns, and let θ^* be the largest principal angle between $\text{col}(\mathbf{M}_s^A)$ and $\text{col}(\mathbf{M}_s^B)$. For every target lead ℓ and observed set S ,*

$$|\eta_{s,\ell}^A(S) - \eta_{s,\ell}^B(S)| \leq 2 \sin(\theta^*/2) (1 + 2 \kappa_s(S)), \quad \kappa_s(S) = \|\mathbf{M}_{s,S}^+\|_2 = \|\mathbf{M}_s \mathbf{M}_{s,S}^+\|_2,$$

the observed-set conditioning of Prop. 1. In particular $\theta^=0$ gives $\eta_{s,\ell}^A(S) = \eta_{s,\ell}^B(S)$ exactly.*

Proof sketch. η is unchanged if $\mathbf{M}_s$ is right-multiplied by any $\mathbf{R} \in O(3)$ (it rotates $\mathbf{P}_{\text{obs}}$ and $\mathbf{e}_\ell^\top \mathbf{M}_s$ together), so align the bases by the optimal $\mathbf{R}$, giving $\|\mathbf{M}_s^A - \mathbf{M}_s^B\|_2 = 2 \sin(\theta^*/2) =: \delta$. With $\mathbf{m}_\ell = \mathbf{M}_s^\top \mathbf{e}_\ell$ ($\|\mathbf{m}_\ell\| \leq 1$) and $\mathbf{P}_S$ the row-space projector, $|\|(\mathbf{I} - \mathbf{P}_S^A) \mathbf{m}_\ell^A\| - \|(\mathbf{I} - \mathbf{P}_S^B) \mathbf{m}_\ell^B\|| \leq \|\mathbf{m}_\ell^A - \mathbf{m}_\ell^B\| + \|\mathbf{P}_S^A - \mathbf{P}_S^B\|_2 \|\mathbf{m}_\ell^B\| \leq \delta + 2\delta \kappa_s(S)$, using the projector perturbation $\|\Delta \mathbf{P}_S\|_2 \leq 2\|\Delta \mathbf{M}_{s,S}\|_2 \|\mathbf{M}_{s,S}^+\|_2$ (Stewart–Wedin) and $\|\mathbf{m}_\ell^B\| \leq 1$. $\square$

What this buys, and its honest limits. The bound is a $\sin \Theta$ perturbation (Davis–Kahan [12]) that makes a *single* extra cohort load-bearing: the transferability of the verdict is governed by a *computable* quantity—the principal angle between the cohorts’ dipolar subspaces—so a large angle *flags* non-transfer before any target-cohort label is seen. The constant $1 + 2\kappa_s$ is worst-case and the bound is loose in absolute terms (for a well-conditioned observed set it can exceed the trivial $\eta \leq 1$); its value is as a *pre-deployment transfer-risk flag*: a small principal angle certifies a small verdict drift, a large one refuses to. On PTB-XL→Chapman (matched sinus, limb-6 → V2) the two regimes are visible. The QRS dipolar subspace is nearly shared ($\theta^*=13.8^\circ$) and the measured drift is correspondingly tight (0.033, realized slope 0.14)—the verdict transfers. The ST subspace is near-orthogonal ($\theta^*=85.7^\circ$, CI up to 89.6°): the bound turns vacuous and the drift is genuinely uncertain—the target-cohort η bootstrap CI is $[0.02, 0.71]$, spanning most of the admissible range, so the realized slope is only pinned to $[0.00, 0.51]$ and we claim *no* precise ST drift, only that the large angle flags the ST verdict as non-transferable (`experiments/transfer_bound.py`). We therefore do not read a stable cross-segment Lipschitz constant from these two points; the load-bearing claim is the rigorous bound plus the angle as a computable transfer-risk diagnostic, corroborated by the tight QRS point. This is the cross-cohort analogue of the finite-sample $\sin \Theta$ term in Thm. 3(iii): both control how a fitted subspace perturbs the verdict.

4 Distribution-Free Calibration

4.1 Calibrated intervals for the predictable residual

The prediction target is the *record-level segment-mean* off-dipole residual of the target lead (one scalar per record: the mean over the segment’s samples). We train a gradient-boosted quantile regressor of it on the observed leads’ segment means and apply Mondrian conformalized quantile regression [10] per (S, s, ℓ) group, with strict fold discipline: basis and model on folds 1–7, the regressor’s `max_iter` selected by pinball loss on fold 8, conformal calibration on fold 9, and a single evaluation on fold 10 (nominal $1-\alpha=0.90$). This is a sound *application* of established conformal machinery, not a new predictor; the contribution is attaching it, per feature and lead, to the recoverability map. Across the pre-registered (S, s, ℓ) groups, fold-10 within-group coverage clusters near nominal (0.87–0.94; e.g. {I,II,V1,V3,V5} QRS/V2 0.91, record-bootstrap CI [0.89, 0.93], mean width 0.150 mV).

What is and is not guaranteed. Mondrian CQR gives *within- (S, s, ℓ) -group marginal* coverage under exchangeability—*not* coverage conditional on diagnosis, which is not a conditioning variable. Diagnostic subgroups are therefore not guaranteed and the smallest ones fall below nominal: the worst is the HYP subgroup at coverage 0.72 ($n=85$), the expected finite-sample behaviour when conditioning on a variable the calibration does not stratify on. We report worst-subgroup coverage rather than hide it under the aggregate band, and we make no distribution-free claim beyond exchangeability with the calibration fold.

5 Experiments

Data. PTB-XL [15] (21,799 twelve-lead records, 100/500 Hz, 10-fold split) with P/QRS/ST/T delineation by NeuroKit2 [16]; the map is a deterministic function of the estimated $\mu_s, \mathbf{M}_s$ (record-level bootstrap CIs), not a held-out evaluation. All table numbers regenerate from result JSON via `paper/emit_baseline_table.py`.

The certificate predicts measured error. On held-out records the theoretical expected ambiguity $a_{s,\ell}$ and the measured per-lead dipolar-projection RMSE are strongly rank- and value-correlated (Spearman 0.81, Pearson 0.93; Fig. 1a), the identifiability verdict $\eta > 0$ predicts a large measured error with AUC 0.87, and κ predicts the residual error *within* the identifiable regime—so the map is not a formal artifact but an operational predictor of what each reconstructor can and cannot recover.

5.1 The recoverability map on PTB-XL

We estimate $\mathbf{M}_s$ (rank 3; the cardiac dipole is 3-D *a priori*, and the cumulative explained variance confirms rank 3 captures the bulk—a rank-sensitivity sweep $r=2 \dots 5$ is reported in `results/recoverability_maps` per segment on the training NORM records, with RECORD-LEVEL bootstrap CIs (resample records, refit $\mathbf{M}_s$; $\varrho=10^{-2}$; Fig. 2). We plot the *normalized* identifiability $\tilde{\eta}_{s,\ell} = \eta_{s,\ell} / \|\mathbf{e}_\ell^\top \mathbf{M}_s\|_2$ —a dimensionless relative unobserved geometric gain (a ratio of L_2 norms, not a variance fraction). A single observed lead leaves every other lead unidentifiable; a dipole-spanning {I,II,V2} or {I,II,V1,V3,V5} makes all targets *exactly* identifiable ($\eta=0$).

The robust flagship is exact. The six limb leads are exact linear combinations of I and II, so $\mathbf{M}_{s,s}$ for limb-6 has *exact* rank 2: precordial ST is *exactly* non-identifiable from limb leads at any SNR, independent of ϱ (Prop. 1).

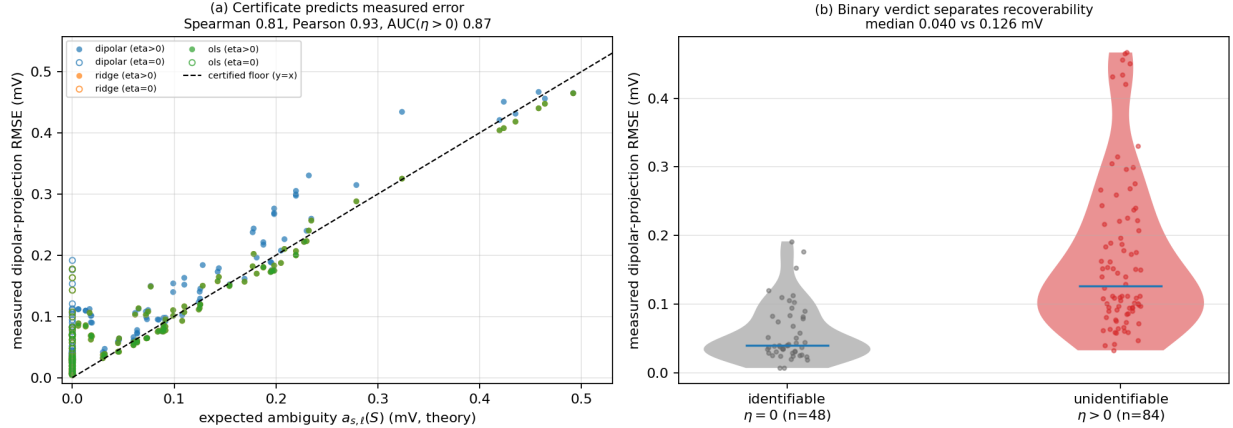


Figure 1: **The certificate is operational.** Across 132 (config, segment, target-lead) cells on held-out fold-10 records, the theoretical per-lead expected ambiguity $a_{s,\ell}(S)$ predicts the *measured* per-lead dipolar-projection error of the dipolar, ridge, and OLS reconstructors. (a) $a_{s,\ell}(S)$ strongly predicts and ranks the measured error (Spearman 0.81, Pearson 0.93). The minimax floor $a_{s,\ell}(S)$ (Thm. 3, $y=x$) is a *worst-case-prior* lower bound, not an empirical envelope: on the true (non-Gaussian) prior, estimators that exploit predictable population structure beat it on a fraction of the 84 non-trivial ($\eta>0$) cells (the pure-dipolar reconstructor on 0.21, ridge/OLS on 0.60)—exactly the Tier-II residual the calibrated intervals of Sec. 4 meter, with the below-floor gap measuring how much structure each exploits. (b) The binary verdict separates recoverability: median dipolar error 0.040 mV for identifiable ($\eta=0$) vs 0.126 mV for unidentifiable ($\eta>0$) leads (AUC 0.87). Regenerated from `results/certificate_validation.json`.

Graded ordering: stable for QRS, weighting-sensitive for ST/T. A duplicated-limb sensitivity analysis (`results/lead_weighting.json`: refit on the 8 algebraically independent leads, on the *same* records) shows the V1–V6 $\tilde{\eta}$ ordering is bootstrap-stable for QRS (Spearman 1.00, CI lower bound 0.88; anterior-before-lateral in 0.98 of resamples) but *not* robust for the low-amplitude ST/T segments (ST Spearman point 0.89 with a wide bootstrap CI; anterior-before-lateral in only 0.77 of resamples). We therefore report the ST/T $\tilde{\eta}$ /ambiguity magnitudes with this caveat and *do not* claim a fine ST/T ordering; the binary limb→precordial verdict above is the robust ST statement. The estimated subspace also depends on diagnosis (a NORM vs. MI principal-angle sensitivity is reported), so the map is a *NORM-trained reference*.

Downstream ST cost across reconstructors. We reconstruct the full 10-second waveform with three real continuous linear reconstructors (observed leads exact; ST at J+60 ms vs. the isoelectric PR-segment baseline [P offset – QRS onset], on fiducials from the observed Lead II shared between truth and reconstruction; median over beats; the PR-segment baseline fell back to the pre-QRS window in $\approx 14\%$ of beats), and count ST-threshold events ($|ST| \geq 0.1$ mV; false positive = crossing in reconstruction not truth, false negative = vice versa; Table 1). On 1498 common records the *total* wrong-event rate is empirically similar across reconstructors (51.1–53.7%) and is *dominated by false negatives* (missed true crossings: limb→precordial recovery smooths ST toward the population), with the false-positive/false-negative split a reconstructor choice (paired record-bootstrap Δ significant). We do not derive a minimax/Bayes bound, so we call the shared total an empirically similar total error rate, *not* a certified lower bound; we report ST-threshold events, not diagnoses.

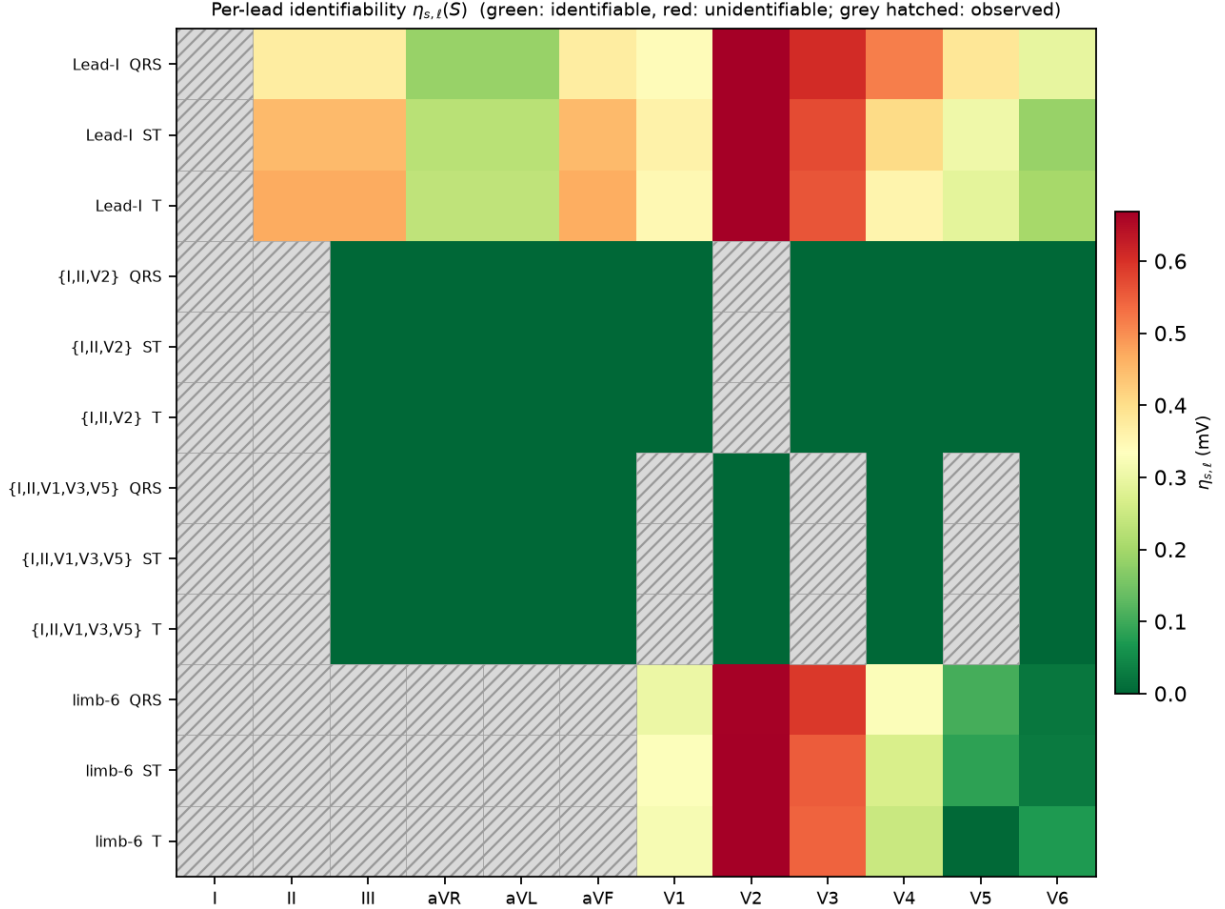


Figure 2: Recoverability map: normalized identifiability $\tilde{\eta}_{s,\ell}(S)$ (left, dimensionless) and prior-conditional expected ambiguity in mV (right), colorblind-safe `cividis`; grey hatched = observed. For limb-6 all precordial leads are unidentifiable ($\eta > 0$); the fine ST/T ordering is weighting-sensitive (text).

Truncation-tolerance sensitivity. Well-posed configurations are ϱ -stable ($\{I, II, V1, V3, V5\}$ is rank 3 across $\varrho \in \{10^{-4}, \dots, 10^{-1}\}$); near-rank -deficient sets are ϱ -sensitive and reported with the ϱ -sweep and bootstrap CIs. A tiny-but-nonzero singular value is exactly identifiable yet ϱ -unidentifiable, which is why we separate exact from ϱ -identifiability (Prop. 1).

5.2 Baselines and what the map predicts

Table 2 reports reconstruction RMSE (mV) of the unobserved target leads under a *single, identical protocol* for all methods: the same shared split (train folds 1–7, ridge λ selected on fold 8, test on NORM fold-10), the same 800 test records, per-timepoint waveform RMSE on the same segment indices, and *paired* record-bootstrap CIs on the step-to-step differences. We include a *representative* masked-completion baseline—an arbitrary-mask conditional 1-D U-Net trained with random lead masks (one model serves any configuration, no target-lead leakage), the same family as [1, 6, 7], run over three seeds (mean reported)—not a state-of-the-art claim.

On a dipole-spanning set the methods form a ladder in point estimates (the ridge $\rightarrow$ U-Net

Table 1: Limb-6 $\rightarrow$ precordial (all V1–V6) ST reconstruction, 1498 common fold-10 records. FP/FN = false-positive/false-negative ST-threshold crossings (%); total = FP+FN. FN dominates (missed crossings); the split is a reconstructor choice. Auto-generated from `results/st_safety.json`.

reconstructor	FP %	FN %	total %	ST err (mV)
dipolar	5.3	45.7	51.1	0.077
ridge	1.7	52.1	53.7	0.076
OLS	1.7	51.8	53.5	0.076

Table 2: Reconstruction RMSE (mV) of target leads, QRS segment, under one identical per-timepoint protocol on NORM fold-10 (800 test records, shared split). “dipolar” = Tier I; “ridge” = per-segment ridge (Tier I+II); “U-Net” = representative masked-completion baseline (3-seed mean). Best in bold. On limb-6 even the neural baseline plateaus far above its identifiable-set error. Numbers auto-generated from `results/fair_baselines.json`.

config	prior-mean	dipolar	ridge	U-Net
{I,II,V2}	0.467	0.236	0.220	0.183
{I,II,V1,V3,V5}	0.481	0.183	0.167	0.164
limb-6	0.481	0.361	0.344	0.238

step is within its paired CI)—prior-mean $\rightarrow$ dipolar (Tier I) $\rightarrow$ per-segment ridge (Tier I+II) $\rightarrow$ U-Net—evidence of recoverable non-dipolar content (the calibrated Tier II layer), though the neural model’s margin over a simple ridge is small (QRS 0.164 vs. 0.167 mV on {I,II,V1,V3,V5}; paired Δ CI $[-0.007, +0.000]$ mV). On **limb-6** the U-Net still *lowers* error over the prior mean (QRS 0.481 $\rightarrow$ 0.238)—it recovers the predictable, largely population-correlated structure—but plateaus at $1.46\times$ its spanning-set error (paired spanning $\rightarrow$ limb-6 Δ CI $[+0.070, +0.079]$ mV) and no reconstructor closes that gap. This residual gap is *consistent with* the unobserved dipolar coordinate the map’s $\eta > 0$ marks as unidentifiable (we do not decompose the error into projected components, so we claim consistency, not equality): capacity recovers the predictable part but not the missing coordinate, so a reconstruction that looked confident about anterior precordial morphology here would be inventing it. We therefore scope the identifiability claim to the dipolar component, not to a blanket “not recoverable.”

Empirical, not physical, subspace. On real PTB-XL the estimated $\mathbf{M}_s$ has three graded principal angles to the classical inverse-Dower vectorcardiographic space [3, 4]: one close ($2\text{--}6^\circ$), one moderate ($10\text{--}15^\circ$), and one substantially different ($43\text{--}55^\circ$, bootstrap CIs excluding small angles). We therefore treat $\mathbf{M}_s$ as an *empirical rank-3 spatial subspace* estimated from the data, not a physical cardiac dipole; all recoverability quantities are stated with respect to this estimated subspace. This empirical, data-driven subspace—rather than a fixed physiological transform—is what makes the map dataset-specific and, we verify, stable across PTB-XL and Chapman for the QRS segment (and robust to the duplicated-limb weighting).

The certificate is prescriptive, not only diagnostic. Because $a_{s,\ell}(S)$ is a function of the chosen leads, it also selects them: minimising the total certified ambiguity $J(S) = \sum_{s,\ell} a_{s,\ell}(S)^2$ over an electrode budget k turns the map into an optimal lead-placement rule. Greedy forward selection matches the *exhaustive* optimum at every budget (over the eight independent measured leads), and the ambiguity reduction is empirically submodular (1.00 of randomized diminishing-returns tests satisfied), so the greedy order carries a $1-1/e$ reading. The prescription is concrete

and non-obvious: the single most informative lead is $V4$, the best pair $\{V4, V2\}$ (certified ambiguity 0.19 vs 0.31 mV^2 for a random pair), and three certificate-chosen leads $\{V4, V2, II\}$ already recover the *entire* dipolar ECG ($J=0$)—whereas random triples need not span the dipole. The marginal value of electrodes is concentrated in the first three, and the certificate says which (`experiments/active_selection.py`).

6 Cross-Dataset Transfer (Matched Comparison)

To test whether the certificate is an artifact of one cohort we re-estimate $\mathbf{M}_s$ under the same unified truncated-SVD numerics on the independent Chapman-Shaoxing-Ningbo 12-lead database [17] and compare principal angles, rank, and κ to PTB-XL. To avoid confounding *cohort* with *case mix*, we run a **matched** comparison (PTB-XL NORM vs. Chapman sinus-rhythm records) alongside an all-record comparison. Chapman records are sampled *randomly* from the complete manifest with a fixed seed (not a deterministic folder/name stride), channels are reordered by WFDB `sig_name` with a 12-lead assertion and resampled from the record’s actual sampling rate, and the exact loaded manifest is hashed (`results/cross_dataset.json`); we used 350 Chapman records, of which 81 were sinus-rhythm.

QRS transfers; ST/T do not. In the *matched* (normal-vs-normal) comparison the QRS subspaces are close (max principal angle 14° , record-bootstrap CI [10, 25]), and the well-posed conditioning transfers (κ for $\{I, II, V1, V3, V5\}$: 2.4 PTB-XL vs. 2.3 Chapman). limb-6 is effective rank 2 on both cohorts (we do not invert a near-zero singular value into a spurious 10^5 -scale κ). The low-amplitude segments differ sharply even under the matched comparison (ST max angle 86° , T 81°), and the all-record comparison is similar (QRS 14° , ST 81°). Because the ST/T difference *survives the matched case mix*, we describe it as cohort-specific: the QRS map and the limb-6 verdict are cohort-stable, while the ST/T third spatial direction is cohort-specific and should be re-estimated per dataset.

Duplicated-limb weighting (`results/lead_weighting.json`). On the *same* records as the primary map, refitting on the 8 algebraically-independent leads leaves the identifiability *verdict* unchanged (spanning-set η identical; limb-6 precordial stays unidentifiable) and the QRS $\tilde{\eta}$ ordering bootstrap-stable, but the low-amplitude ST/T ordering is *not* robust to the weighting (Sec. 5); we therefore make no fine ST/T ordering claim under either fit.

7 A Negative Result: the Generative “Fabrication-Objective” Story Did Not Survive

This project began with a stronger claim—that a diffusion reconstructor proves “fabrication is a property of the objective,” a certified hallucination. A pre-submission re-analysis abandoned it. We document the abandonment in full because the negative result is itself informative and because selective reporting of the earlier (favourable) numbers would be exactly the failure the certificate is meant to prevent.

Setup. An arbitrary-mask conditional DDPM (12-channel binary mask + observed values; random-mask training so one model serves any configuration, with explicit leakage tests) reconstructs $\{V2, V4, V6\}$ from $\{I, II, V1, V3, V5\}$ under classifier-free-style guidance weight $w \in \{1, 2, 4, 6\}$ (`results/gpu_deficit_ci.json`, `realism_metrics.json`, `gpu_oracle_gate.json`; all lineage-stamped, numbers auto-generated).

(1) The original effect was a configuration-leakage artifact. The earlier exhibit reused one model trained on a fixed observed set to score a *different* configuration. With leakage removed

(arbitrary-mask model + leakage guards) and 5 training seeds, the generative reconstructor does *not* outperform a simple held-out *ridge achievability comparator* (a ridge fit on the observed leads, used as an achievability reference—*not* a Bayes-optimal oracle, since the model can and does exceed it at low guidance). The deficit $\Delta\rho = \rho_{\text{ridge}} - \rho_{\text{model}}$ —how far the model’s target-lead correlation falls below the ridge comparator ρ_{ridge} —is *monotone* in w : mean $\Delta\rho = -0.026, -0.006, +0.041, +0.072$ for $w = 1, 2, 4, 6$ (mean over 5 seeds; SD ± 0.044 at $w=6$). It is small and negative at $w=1$ (the model marginally beats the comparator) and grows to a modest but seed-*significant* $+0.072$ at $w=6$ (95% t -interval $[+0.011, +0.134]$, *excludes zero*; the model now falls short of the comparator). Guidance thus moves the gap the *wrong* way for the fabrication story: it does not buy achievability beyond a plain ridge, it erodes the model’s small low-guidance edge and drives it significantly below the comparator. The previously reported large diffusion advantage does not reproduce.

(2) Realism does not improve with guidance—the causal premise is false. The story required higher guidance to buy realism at the cost of fidelity. Instead realism is *no better* at high guidance: from $w=1$ to $w=6$ the target-lead PSD log-distance rises (V2: $1.88 \rightarrow 2.25$; a small dip at $w=2$ then a clear rise, so worsening but not monotone) and the amplitude-Wasserstein distance rises (V2: $0.45 \rightarrow 0.83$). The trend is toward *worse* realism on essentially every target-lead axis (both PSD log-distance and amplitude-Wasserstein rise from $w=1$ to $w=6$); there is no monotone realism/fidelity trade to exploit. This is consistent with fundamental limits on hallucination in inverse problems [18] and the perception–distortion tradeoff [19]: raising perceptual pressure (guidance) cannot manufacture target-lead information the observed leads do not carry.

(3) What is true instead. The predictable non-dipolar residual is real but modest: ridge-comparator correlations (`gpu_oracle_gate.json`) are $\rho \approx 0.49\text{--}0.55$ for V2 (QRS/ST) and $0.21\text{--}0.34$ for V4/V6—consistent with the calibrated Tier-II intervals of Sec. 4. So the honest statement is a *graded predictability* of the residual, calibrated per feature and lead, not a proof of fabrication. We removed the fabrication-objective claim and rebuilt the paper around the parts that withstand scrutiny.

(4) A rigorous, ground-truth-free fabrication audit. The certificate lets us make the fabrication idea precise without ground truth. Whatever a reconstruction asserts in the *unidentifiable* dipole subspace $\mathbf{Q} = \mathbf{I} - \mathbf{P}_{\text{obs}}$ is content the observed leads cannot fix; we score the fraction of a reconstruction’s dipolar energy that lies there, $\phi = \|\mathbf{M}_s \mathbf{Q} \hat{\mathbf{d}}\|^2 / \|\mathbf{M}_s \hat{\mathbf{d}}\|^2$ (Fig. 3). Principled reconstructors *abstain*: the dipolar and prior-mean reconstructions have $\phi=0$ everywhere (≤ 0.000), and even ridge/OLS keep $\phi=0$ wherever the observation spans the dipole (limb-6, {I,II,V2}). But on a heavily under-determined single lead the linear reconstructors fabricate ($\phi = 0.50$ for ridge, Lead-I QRS), and the diffusion model fabricates most of its dipolar content in the provably unrecoverable subspace ($\phi \approx 0.75$) with fabrication *rising with guidance* ($\phi: 0.75 \rightarrow 0.84$ QRS, $\rightarrow 0.86$ ST as $w:1 \rightarrow 6$; $\phi \leq 0.00$ on limb-6). This is the defensible, disciplined form of “fabrication is a property of the objective”: more guidance manufactures more content the data cannot support, without improving achievability (point 1)—quantified by the certificate rather than asserted.

8 Limitations

The subspace $\mathbf{M}_s$ is estimated, so η/κ inherit its estimation error (reported via record-bootstrap CIs and the rank/ ϱ /subspace-angle sensitivities of Fig. 4) and its cohort dependence (Sec. 6); the ST/T third direction is cohort-specific. That cohort dependence is itself bounded: Thm. 4 shows the verdict drifts across cohorts only as fast as the two cohorts’ dipolar subspaces separate ($\sin \Theta$), so the measured principal angle is a computable transfer-risk flag—small for QRS (13.8° , tight measured drift 0.033) and large for ST (85.7° , where the bound is vacuous and the drift itself is

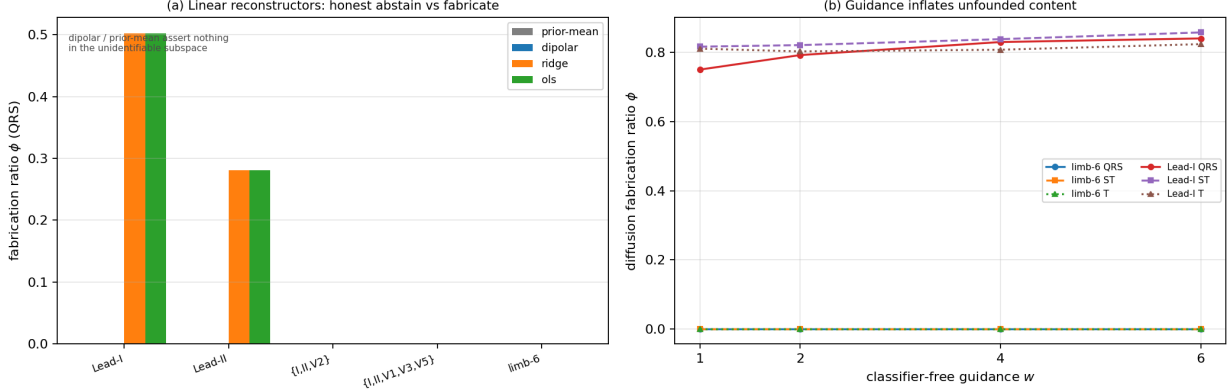


Figure 3: **Null-space dipolar fabrication audit** (ground-truth-free). ϕ = fraction of a reconstruction’s dipolar energy in the certified-unrecoverable subspace $\mathbf{Q}$. (a) Linear reconstructors: dipolar/prior-mean abstain ($\phi=0$); single-lead ridge/OLS fabricate; on observation sets that span the dipole (incl. limb-6) even ridge/OLS abstain. (b) The diffusion model fabricates most of its dipolar content on a single lead and fabrication *grows with classifier-free guidance* w , while abstaining on limb-6. Regenerated from `results/fabrication_audit.json`, `fabrication_diffusion.json`.

uncertain), flagging the ST verdict as non-transferable in advance. Identifiability is stated for the low-rank (dipolar) coordinate: a reconstructor may still recover predictable non-dipolar structure (Sec. 5), so $\eta > 0$ bounds the dipolar component, not every waveform detail. Conformal coverage is within- (S, s, ℓ) -group marginal under exchangeability, not conditional on diagnosis; rare diagnostic subgroups are under-covered (Sec. 4). Delineation uses NeuroKit2 and inherits its errors, but the ST-safety endpoint is robust to it: across a delineator $\times$ rate grid ($\text{dwt}/\text{cwt} \times 100/500\text{Hz}$) the precordial-not-recoverable verdict is sign-stable ($\eta > 0$ at every setting, as forced by the structural rank-2 limb subspace), the per-lead recoverability ordering is preserved (Spearman ≥ 0.94), the rate axis is tight (strong-lead $|\Delta\eta| \leq 0.028$), and the endpoint stays FN-dominated (under-warning, the conservative direction) in every setting (`results/delineator_robustness.json`). The certificate is a pre-reconstruction screen, not a substitute for clinical validation.

9 Conclusion

Reduced-lead ECG reconstruction has a known low-rank structure per segment. Exploiting it gives a graded, per-target-lead recoverability map—what is identifiable, how well-conditioned, and (via calibration) what interval the predictable residual admits—before and independent of any reconstructor, plus a total ST-threshold error rate that is empirically similar across the evaluated reconstructors. It converts “trust the reconstruction” into a per-feature, per-lead, calibrated statement, and it told us honestly when one of our own earlier claims was not supported.

A Proof of Proposition 1

We work with the *exact* $\mathbf{M}_{s,S} = \text{Sel}_S \mathbf{M}_s$, its exact Moore–Penrose inverse $\mathbf{M}_{s,S}^+$, and the exact orthogonal row-space projector $\mathbf{P}_{\text{obs}} = \mathbf{M}_{s,S}^+ \mathbf{M}_{s,S}$ (projecting onto $\text{row}(\mathbf{M}_{s,S}) = \ker(\mathbf{M}_{s,S})^\perp$).

(i) **Identifiability.** The dipolar part of lead ℓ is the linear functional $f_\ell(\mathbf{d}) = \mathbf{e}_\ell^\top \mathbf{M}_s \mathbf{d}$. From S we observe $\mathbf{M}_{s,S} \mathbf{d}$, which fixes $\mathbf{P}_{\text{obs}} \mathbf{d}$ exactly; the exact kernel component $(\mathbf{I} - \mathbf{P}_{\text{obs}}) \mathbf{d}$ is unobserved.

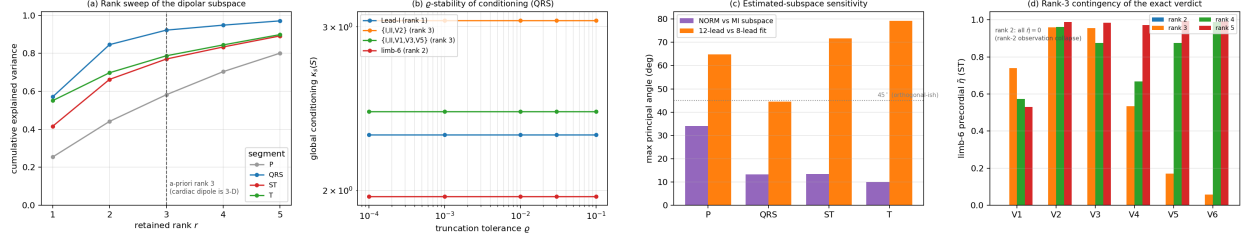


Figure 4: Sensitivities of the certificate. (a) Rank sweep: cumulative explained variance of the per-segment dipolar subspace; the *a-priori* rank-3 choice (the cardiac dipole is 3-D) captures the bulk (QRS/ST). (b) ρ -stability: global conditioning $\kappa_s(S)$ is flat in the truncation tolerance for the deployed observed-lead sets (near-rank-deficient sets, reported in `results/recoverability_maps.json`, are ρ -sensitive). (c) Estimated-subspace sensitivity: max principal angle per segment under a NORM→MI diagnosis shift and under the 12-vs-8-lead weighting; the low-amplitude ST/T subspaces move most (near/above 45° for the weighting), which is why we make no fine ST/T ordering claim. (d) Rank-3 contingency of the exact limb-6 verdict: at retained rank 2 the limb-6 observation is itself rank-2, so all precordial $\tilde{\eta}=0$ (a spurious “identifiable” collapse); the precordial non-identifiability appears only at rank ≥ 3 , so the exact verdict is exact *given* the a-priori rank-3 dipole model.

Decompose $\mathbf{e}_\ell^\top \mathbf{M}_s = \mathbf{e}_\ell^\top \mathbf{M}_s \mathbf{P}_{\text{obs}} + \mathbf{e}_\ell^\top \mathbf{M}_s (\mathbf{I} - \mathbf{P}_{\text{obs}})$. If $\eta_{s,\ell}(S) = \|\mathbf{e}_\ell^\top \mathbf{M}_s (\mathbf{I} - \mathbf{P}_{\text{obs}})\|_2 = 0$ then $f_\ell(\mathbf{d}) = \mathbf{e}_\ell^\top \mathbf{M}_s \mathbf{P}_{\text{obs}} \mathbf{d}$ depends on $\mathbf{d}$ only through the observed $\mathbf{P}_{\text{obs}} \mathbf{d}$, hence is determined by $\mathbf{M}_{s,S} \mathbf{d}$: identifiable. Conversely if $\eta_{s,\ell}(S) > 0$ there is a unit $\boldsymbol{\delta} \in \ker \mathbf{M}_{s,S}$ with $\mathbf{e}_\ell^\top \mathbf{M}_s \boldsymbol{\delta} \neq 0$; then $\mathbf{d}$ and $\mathbf{d} + t\boldsymbol{\delta}$ give identical observations for all t but change f_ℓ by $t \mathbf{e}_\ell^\top \mathbf{M}_s \boldsymbol{\delta}$, so no estimator recovers f_ℓ at any SNR. This is the exact estimability criterion [8] for f_ℓ .

Exact vs. ρ -truncated verdict (numerical check). The deployed heatmap replaces $\mathbf{P}_{\text{obs}}$ by the rank- r truncation $\mathbf{P}_{\text{obs}}^{(\rho)} = \mathbf{V}_r \mathbf{V}_r^\top$ (singular directions with $\sigma_i \leq \rho \sigma_{\max}$ dropped). This can disagree with the exact verdict only near a truncation boundary, and only in the *conservative* direction: $\eta_{s,\ell}^{(\rho)} \geq 0$ can be positive where $\eta_{s,\ell} = 0$ (a tiny-but-nonzero singular value is exactly identifiable yet not stably recoverable at resolution ρ), but not vice versa. We verify all three regimes in code (`tests/test_exact_vs_rho.py`): (a) an exactly rank-deficient $\mathbf{M}_{s,S}$ gives $\eta = \eta^{(\rho)} > 0$; (b) $\mathbf{M}_{s,S} = \text{diag}(1, 1, 10^{-6})$ gives $\eta = 0$ but $\eta^{(\rho)} > 0$ at $\rho = 10^{-2}$; (c) a full-rank well-conditioned $\mathbf{M}_{s,S}$ gives $\eta = \eta^{(\rho)} = 0$. The limb-6 flagship falls in regime (a) exactly (rank 2), so its verdict is ρ -independent.

(ii) **Conditioning.** The mean-centred estimate is $\hat{\mathbf{L}}_s - \boldsymbol{\mu}_s = \mathbf{M}_s \mathbf{M}_{s,S}^+ (\mathbf{y}_S - \boldsymbol{\mu}_{s,S})$ with $\mathbf{y}_S - \boldsymbol{\mu}_{s,S} = \mathbf{M}_{s,S} \mathbf{d} + \text{Sel}_S \mathbf{r}_s + \mathbf{n}$. On the identifiable part its lead- ℓ error from the observed non-dipolar residual and noise is $\mathbf{e}_\ell^\top \mathbf{M}_s \mathbf{M}_{s,S}^+ (\text{Sel}_S \mathbf{r}_s + \mathbf{n})$, and by Cauchy–Schwarz $|\mathbf{e}_\ell^\top \mathbf{M}_s \mathbf{M}_{s,S}^+ (\text{Sel}_S \mathbf{r}_s + \mathbf{n})| \leq \|\mathbf{e}_\ell^\top \mathbf{M}_s \mathbf{M}_{s,S}^+\|_2 (\|\text{Sel}_S \mathbf{r}_s\|_2 + \|\mathbf{n}\|_2) = \kappa_{s,\ell}(S) (\|\text{Sel}_S \mathbf{r}_s\|_2 + \|\mathbf{n}\|_2)$, using Cauchy–Schwarz and the triangle inequality. Each per-lead conditioning is a row norm of $\mathbf{A} = \mathbf{M}_s \mathbf{M}_{s,S}^+$, so $\kappa_{s,\ell}(S) = \|\mathbf{e}_\ell^\top \mathbf{A}\|_2 \leq \|\mathbf{A}\|_2 = \kappa_s(S)$; the global κ_s upper-bounds every per-lead value but is not their maximum. $\square$

B Reproducibility

Code, fixed seeds, an exact environment lock (`env.lock.txt`), and a one-command pipeline (`experiments/run_all`) are released; every table number is regenerated from result JSON (`paper/emit_baseline_table.py`), and a CI workflow runs the test suite on every push. The neural baseline is a 24→12-channel 1-D

U-Net (base width 64, residual GroupNorm blocks, 60 epochs, 3 deterministic seeds, Adam 2×10^{-4} , MSE on masked leads, random-mask training); the residual predictor is a gradient-boosted quantile regressor (pinball loss) with strict fold discipline (folds 1–7 train, 9 calibrate, 10 test).

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